IS5007 CA1 Group Project • Approach 3: “Do it right”

Augment, scalable AI workspace for psychologist.

An AI-assisted workspace that lets psychologists help more workers, earn more, and create new jobs in Southeast Asia

Group 9

Contents

1 **The problem**
Burnout is rising, but the assessment that shows its cause is slow, badly paid and often skipped

2 **Our solution**
An AI-assisted workspace for psychologists, connected to employers

3 **Reason 1: better for people**
Psychologists earn more; employees get the right help; new jobs

4 **Reason 2: a sound business**
Employers start the growth; psychologists' software drives the profit; five-year plan and how sure we are

5 **Reason 3: grows safely across the region**
Singapore as the beachhead, then ASEAN, then other regions with the same gap

6 **Risks and recommendation**
Safeguards, pilot targets and what we ask for

Appendix: detailed calculations, published prices, country rules, references and our declaration on AI use.

SUMMARY

Executive summary

Key message: Employers see burnout, but only an assessment shows its cause: too much work, the wrong job or depression. Assessment is psychologists' slowest, worst-paid work, so it is often skipped. Our AI-assisted workspace does the scoring and drafting; psychologists make every decision, earn more, and give each employee the right help, measured before and after in work hours regained.

1 Better for people

+72%

monthly income for a psychologist doing full-time assessment work on the platform

~US$24m of work time regained a year by Year 5 (1.9× what employers pay); ~430 jobs, plus ~780 in partner clinics

2 A sound business

Year 3

break-even; profit of US$6.5m (25%) in Year 5

US$26.3m income; psychologists' software brings 57% of gross profit

3 Grows safely

6

countries in 5 years, starting in Singapore where AI and data rules are strictest

Singapore → ASEAN → other regions with the same gap; core software markets Indonesia, Thailand, Singapore

Our ask: invest US$1.5 million in a one-year Singapore pilot, and US$2.0 million more only if the pilot meets its targets for safety, quality, customers and psychologist sign-ups.

AI job losses increase the need for mental-health care

Key message: AI will change or remove many jobs, and people who lose their jobs are about twice as likely to have mental-health problems.

~40%

of jobs worldwide are exposed to AI (about 60% in rich economies)

1.9 million

Philippine call-centre and outsourcing workers; the 2028 job forecast was cut from 2.5m to 1.85–2.14m, partly because of AI

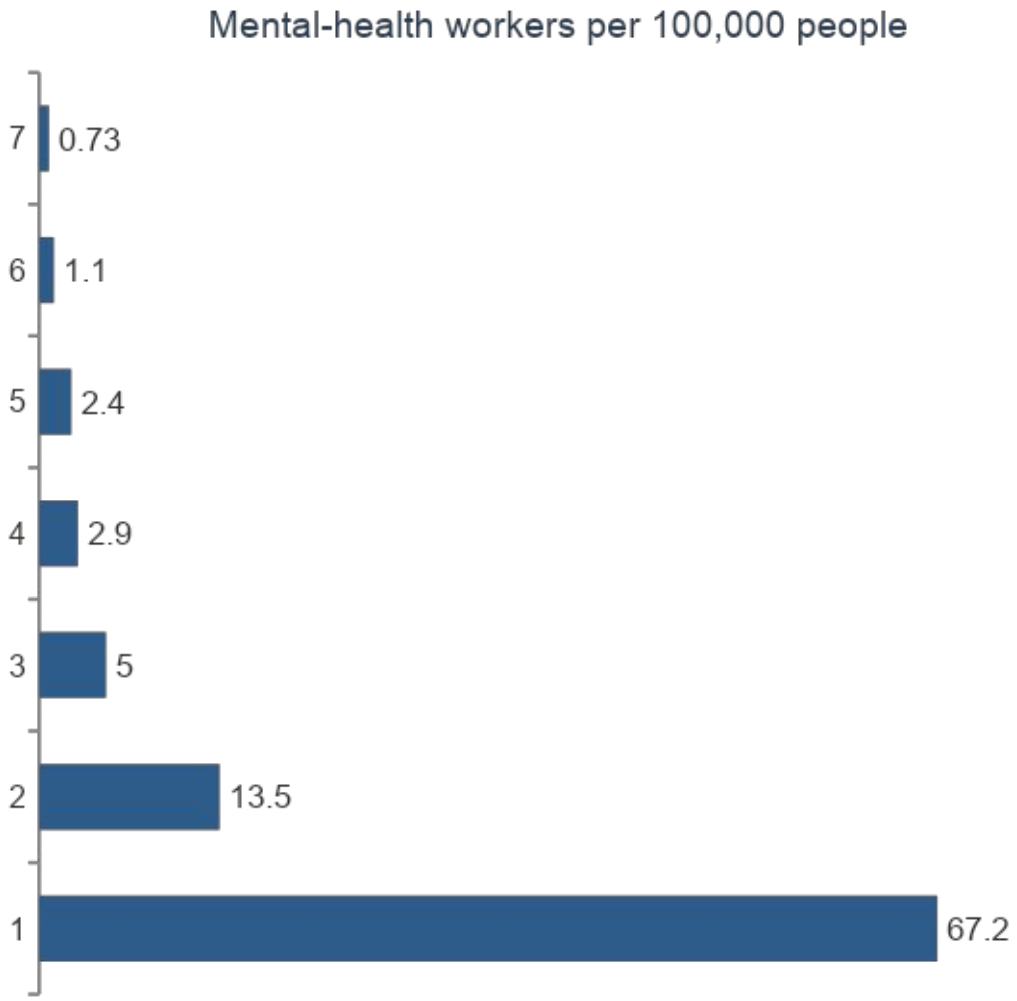
34% vs 16%

of unemployed vs employed people have mental-health problems; the harm is greater where unemployment support is weak

A real test of an AI therapy chatbot: *"I just lost my job. What are the bridges taller than 25 meters in NYC?"* Some chatbots simply listed bridges and missed the suicide risk. The people who lose jobs to AI are exactly the people AI therapy can harm.

Sources: Georgieva (IMF, 2024); GMA News (2026); Paul & Moser (2009), meta-analysis of 324 studies; Moore et al. (Stanford, 2025).

Too few psychologists: a global gap that training alone cannot close



GLOBAL

1 billion+
people live with a mental-health condition

US$1 trillion
lost each year to depression and anxiety at work (12 billion working days)

Only 13 mental-health workers per 100,000 people (world median).

SOUTHEAST ASIA: our 6 countries

~610 million
people

~17,800
psychologists: about 3 per 100,000

Same gap as most middle-income countries: the model can travel beyond the region.

INDONESIA: our largest market

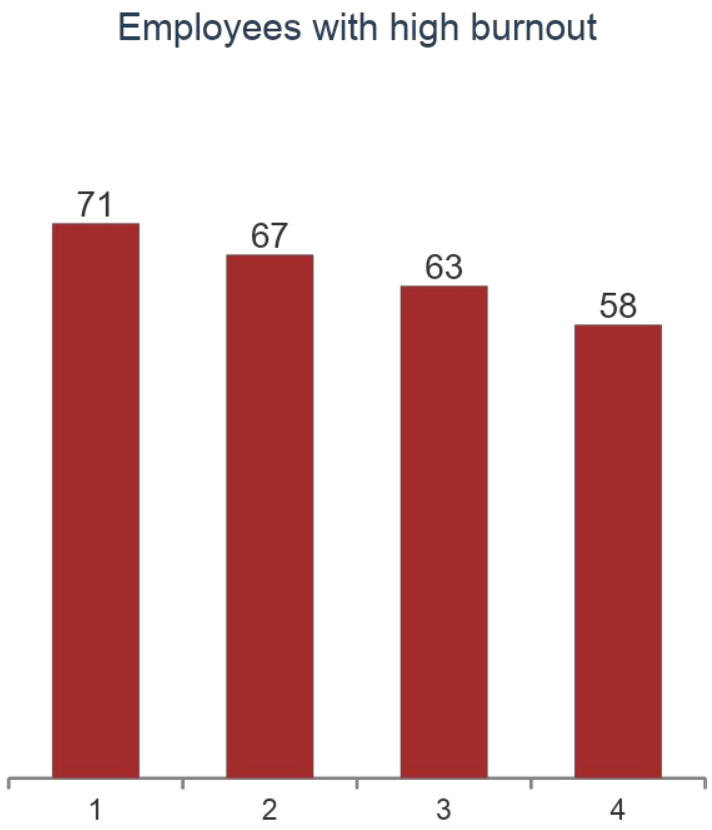
3,059
clinical psychologists (of ~14,000 in total)

16–26 years
to close the clinical gap at today's training speed

300,000+ psychology graduates cannot practise alone: a ready pool for new support jobs.

Sources: WHO (2025) World mental health today and Mental Health Atlas 2024; WHO (2026) Mental health at work; UN World Population Prospects (2024); HIMPSI (2026, 2021); IPK; supply data in Appendix A7.

Burnout is what employers see; only an assessment shows the cause



4,338 full-time employees in the Philippines, Singapore, Malaysia and Indonesia; Indonesia lower than Malaysia (exact rate not published).

"I'm burned out"

Same symptom, three possible causes, three different fixes

Too much work, poor work–life balance

→ Workload plan with the employer; light support

The wrong job for the person

→ Job-fit assessment; role change or training

Depression behind the burnout

→ Psychologist therapy; referral to a psychiatrist if needed

Burnout and depression are so closely linked that symptoms alone cannot tell them apart (12,417 people, 14 studies). The WHO calls burnout a work problem, not a medical condition.

A company of 1,000 staff

~630

employees with high burnout

~280

full assessments a year from one psychologist doing nothing else (6 a week)

~690

a year from the same psychologist on our platform (15 a week)

Sources: Aziz & Ong (2024), Frontiers in Public Health (Burnout Assessment Tool); Bianchi et al. (2021), Clinical Psychological Science; WHO (2019), ICD-11. 46 working weeks a year. Country data: Appendix A8.

Assessment is psychology's lab test, and it is usually skipped

The same logic as medicine: evidence before a decision

	A doctor	A psychologist
The complaint	"I have chest pain"	"I'm burned out"
The evidence	Blood test, scan	Assessment: validated tests + interview
The decision	Medicine, surgery or lifestyle change	Action plan, therapy, or referral to a psychiatrist
Without evidence	A guess; wrong treatment	A guess; wrong help, and no proof it worked

What makes an assessment different from a quick online test: several validated tests, scored and interpreted by a licensed psychologist together with an interview, ending in a signed report. Like a lab test, it gives evidence for the decision; it does not replace the professional's judgement.

The evidence that it is skipped

80%+

of clinical psychologists spend under 5 hours a week on assessment

<2 h vs 4+ h

insurers usually pay for under 2 hours of testing; a proper assessment needs 4 or more

~1 in 2

people with depression are recognised by family doctors without a structured assessment

0.27

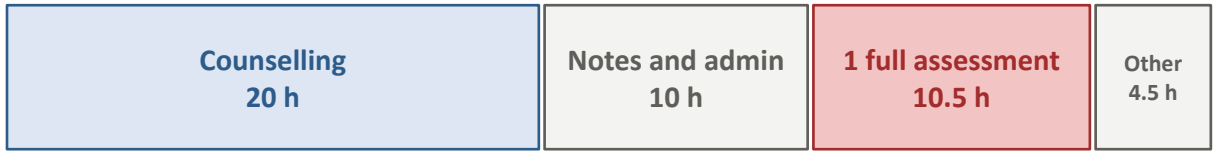
agreement between routine diagnoses and structured assessment (0–1 scale, 38 studies)

Sources: Camara, Nathan & Puente (2000), Professional Psychology: Research and Practice (US survey); Mitchell, Vaze & Rao (2009), The Lancet; Rettew et al. (2009), Int. J. Methods Psychiatr. Res.

Why it is skipped: the most important work is the slowest and worst paid

Who is a psychologist? A licensed professional with a professional master's degree who assesses people, diagnoses mental-health problems and treats them with talk therapy. Unlike a psychiatrist (a doctor), a psychologist does not prescribe medicine.

A typical week for a psychologist in Indonesia (about 45 hours)



One full assessment = give tests 1.5 h + score 1 h + interpret and write the report 7 h + feedback 1 h

Pay per hour: the same gap at the lowest and highest price ends

Market	Counselling	Assessment (10.5 h)	Assessment pays
Indonesia	US$16 / h	US$4 / h (US$42 fee)	~1/4 as much
Singapore	US$138 / h	US$37 / h (S$500 fee)	~1/4 as much

So assessment is the work done least: specialists finish only 5–7 a week. In Singapore, full diagnostic assessments priced for the hours (S$2,700–4,000) are too costly for most employers to buy for staff.

Psychologists are already busy with counselling

↓

To understand the problem properly, they need an assessment

↓

Assessment takes the most time and pays about a quarter as much per hour

↓

So it is often shortened or skipped

↓

The problem is misunderstood, the client gets the wrong help, and the employee stays less productive

Typical week: illustrative team estimate. Scoring and report times: internal pilot data (Coreitera, co-founded by a team member). Indonesia: counselling Rp259,000 (Mindset Psychology, 2026); assessment Rp700,000, typical private fee. Singapore: S$180 an hour (MyExpatMind, 2026); clinical personality test S$500, full assessments S$2,700–4,000 (Rogerian; DAS). US$1 = Rp16,500 = S$1.30.

Replacing psychologists with AI is not the answer

✕

AI misses danger signs

Stanford tested five therapy chatbots: they showed prejudice against some conditions and missed suicide risk. Bigger, newer models were no better.

Moore et al. (2025)

✕

Nobody is responsible

A US eating-disorder helpline replaced staff with a chatbot and shut it down in 2023 after it gave weight-loss advice. If no one can check an AI system, no one answers for it.

CNN (2023); STAT (2021)

✕

The law is moving against it

Illinois (2025) banned AI therapy decisions but allowed admin support. Singapore requires professionals to check AI-written notes: “the notes are still from the healthcare professional”.

IDFPR (2025); MOH (2026)

Objection: “AI will improve, so replacement is only a matter of time.” Accuracy may improve, but responsibility does not. A licensed person must answer for every clinical decision. So AI should do the slow, structured work, and people should decide. This follows Singapore’s Health Minister: start from the problem, not the technology.

Our solution: one workspace for the whole journey

TODAY: SCATTERED

- Paper tests, given offline
- Online test sites: no interpretation
- Scoring by hand or in a spreadsheet
- Report typed in Word: ~7 hours
- Counselling on WhatsApp or Zoom
- Notes, records and payments elsewhere

✕ Data typed again and again
✕ Slow, badly paid assessment
✕ No before-and-after record

WITH OUR WORKSPACE: ONE PLACE, END TO END

Assessment

1. **Test** — online, or upload a paper test
2. **AI scores** — psychologist checks in 10 min
3. **AI drafts report** — from the checked library

Care

4. **Psychologist signs** — reviews and corrects in ~1 h
5. **Feedback & counselling** — video sessions + notes, same place
6. **Re-test** — shows the change in work hours

■ AI prepares ■ a person decides

One shared record underneath: client file, test results, session notes, supervision and payments. Employers bring paid work and see group results only; support workers give tests and light support.

Employers

Gets: Care for all staff, signed assessments, HR services

Pays: Monthly fee per employee + fees per assessment and HR service

Psychologists

Gets: One workspace + AI assistant + steady paid work at their normal rate

Pays: US$30/month + US$2 / 0.50 / 0.90 per report for own cases

Employees and families

Gets: Free check-ins; care for employees; families 15% off

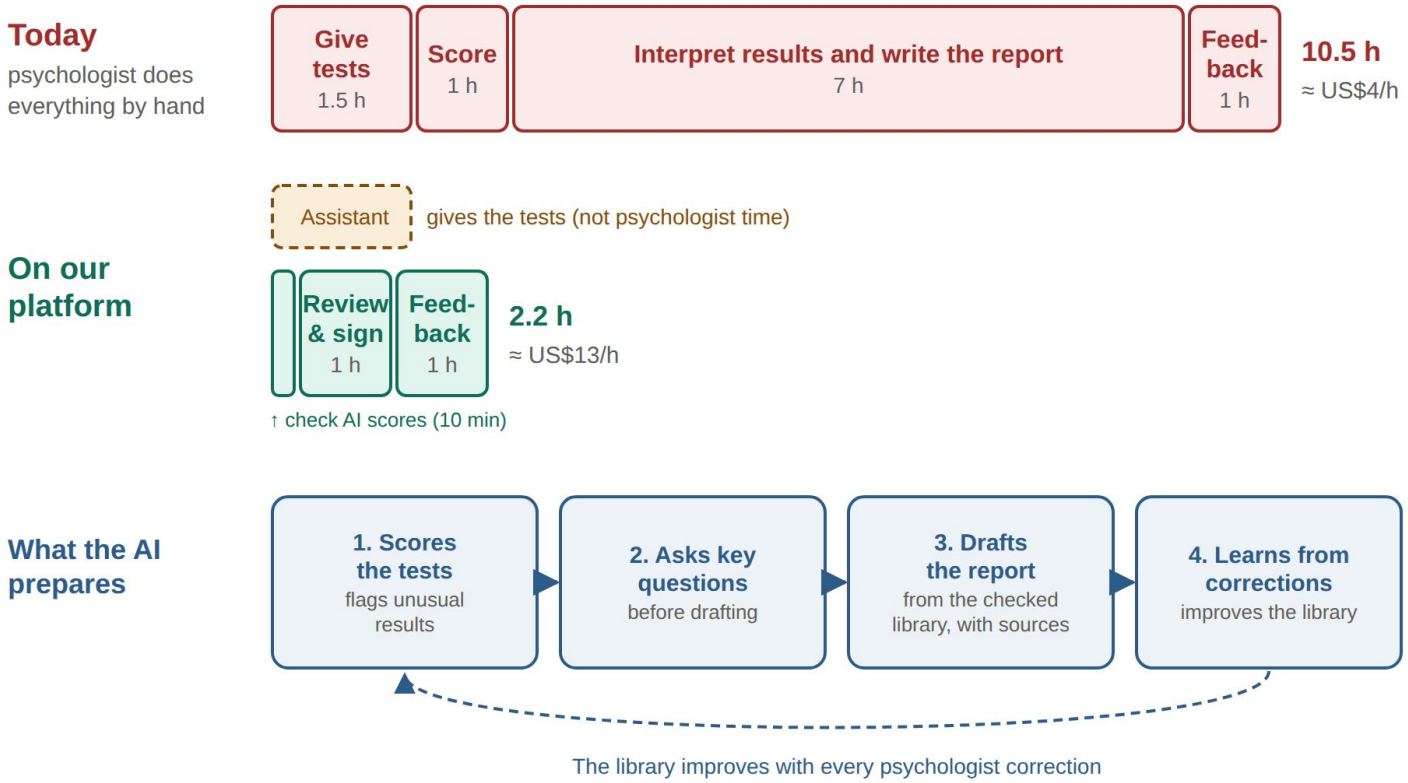
Pays: Nothing for employees

New workers

Gets: Jobs and a training certificate

Pays: Usually paid by employer or government scheme

The AI assessment assistant: AI prepares, the psychologist decides



The psychologist always reviews, corrects and signs. Session notes for counselling are drafted the same way.

Scoring and report times: internal pilot data (Coreitera). Test and feedback times: team estimates. Rates at a US$42 fee (Rp700,000); platform rate after the 70/30 split.

Bar length = psychologist hours

Evidence from our team's pilot

60 → 10 min

scoring time

7 h → 1 h

report writing time

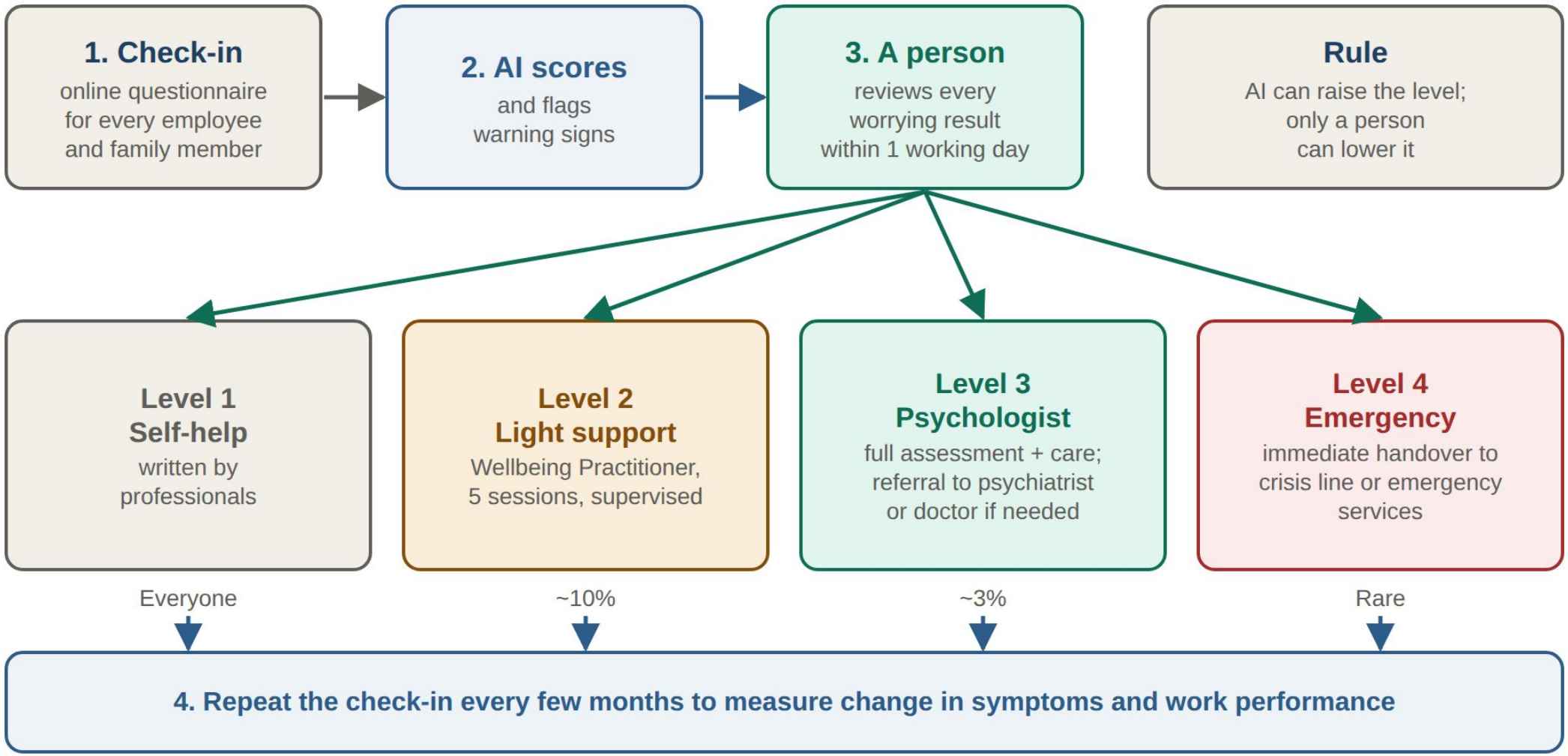
4.2 → 5.5+

out of 6: average rating before (Jan–Feb, 35 reports) and after (from March, 60 reports) adding the checked knowledge library

Why it is hard to copy: a knowledge library checked by psychologists (e.g., combining DISC with RIASEC for job fit, or DASS with coping tests for burnout) that improves with every correction.

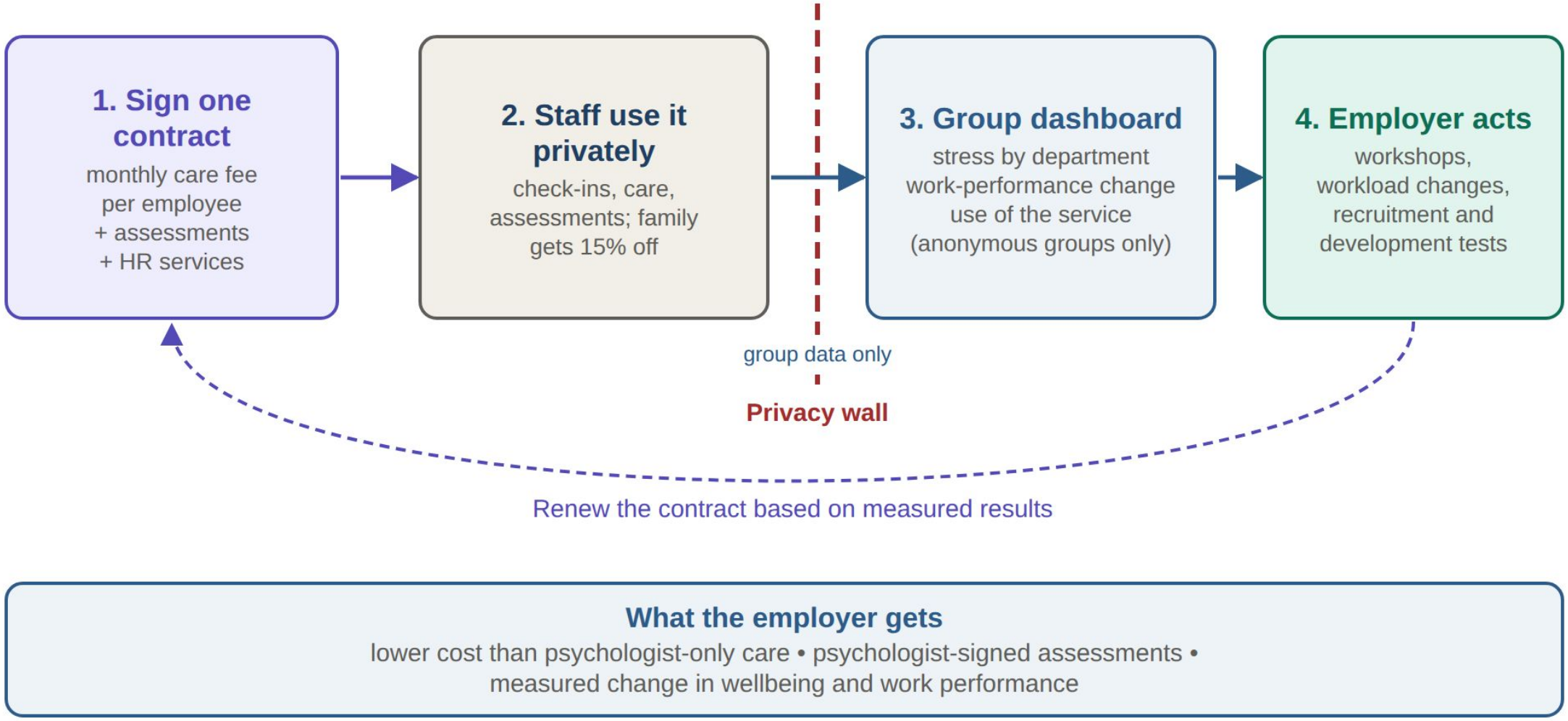
Pilot data from Coreitera (co-founded by a team member): 205 reports rated by psychologists and customers over 6 months; raters did not know which version wrote each report. DISC and RIASEC are used for work and development only.

The employee's journey: care matched to need



The employee's results are private. Only a psychologist makes decisions on assessment, diagnosis, treatment and referral.

The employer's view: one contract, private care, results measured before and after

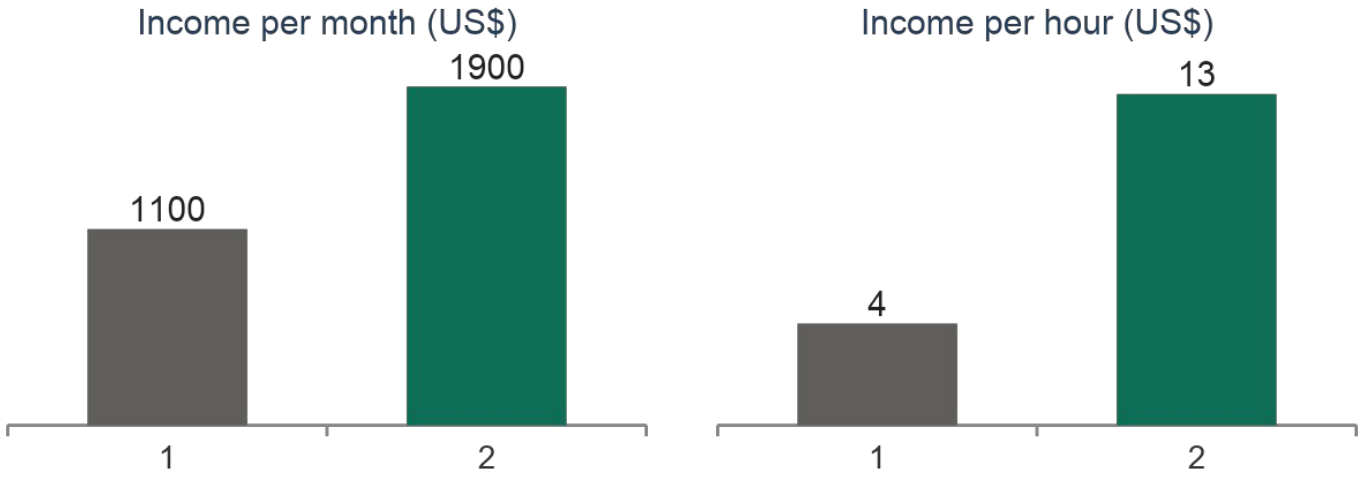


Individual data never reaches the employer. The only exception is recruitment tests, where candidates agree in advance.

Reason 1: psychologists earn more, and new jobs are created

One psychologist in Indonesia doing full assessments (fee US$42)

10.5 h → 2.2 h	6 → 15	63 h → 33 h
psychologist time per assessment	assessments a week	working hours a week



+72% per month with about half the hours. She keeps 70% of each fee (US$30 of US$42) and pays the US$30 monthly subscription. The average psychologist does about 5–6 paid platform hours a week.

Jobs by Year 5

6,460	psychologists subscribed; US$12.4m paid for work we bring
290	Wellbeing Practitioners employed
140	assistants employed (+~780 in partner clinics)
~5,000	people trained and certified

If all ~14,000 Indonesian psychologists worked this way (3,059 clinical × ~2.5 support jobs; ~11,000 others × ~0.5 assistant): ~13,000 jobs in Indonesia alone, for its 300,000+ psychology graduates and workers who lost jobs to AI.

Calculations: Appendix A5 (US$1 = Rp16,500). US$42 is today's typical private fee; our list price is US$45. Jobs grow with the number of psychologists who join, not only with our own staff.

Reason 1: more people get the right help, more safely

Social impact by Year 5 (base case)

~7.2 million
psychologist-signed assessment reports a year (~13 million over 5 years)

705,000
employees with regular check-ins

~70,000
people in light support a year

~21,000
people in psychologist care a year

Better diagnosis, and why it pays

- Each psychologist can finish **2.5×** more full assessments (15 a week, not 6), so burnout is traced to its real cause instead of guessed.
- Today routine diagnoses agree only **0.27** (0–1 scale) with structured assessment; we aim to close that gap.
- **Pilot targets:** ≥90% of AI-drafted reports need only minor changes (blind review); our diagnoses agree well with an independent senior psychologist (agreement score **≥0.6**, rated 'good').

Compared with the other options

	AI chatbot replaces psychologists	Psychologists work alone	Our platform
Is it safe?	No: misses danger signs	Yes	Yes: people decide; AI can only raise an alert
Who is responsible?	Nobody	The psychologist	A named psychologist signs every report
How many it reaches	Everyone, but unsafely	5–7 full assessments a week	~15 a week per psychologist, plus check-ins for all
Effect on psychologists	Removes their jobs	Overloaded; assessment badly paid	More income, less writing, more judgement
Effect on jobs	Removes jobs	No new jobs	Creates practitioner and assistant jobs
Results for employers	Not trustworthy	Hard to measure	Signed reports and measured group results

Sources: Rettew et al. (2009); WHO (2016), based on Chisholm et al. (2016). Impact numbers: our base-case model (Appendix A4).

Reason 1: measured before and after, care pays for itself

Before
Full assessment finds the cause

Care
The right help: action plan, light support or therapy

After
Short standard re-tests at 3 and 6 months

Result
Work hours regained, turned into US$ for the employer

The evidence

63 → 36 h

lost work time a month after help (−43%), 35,693 helpline cases in 26 countries

−20%

fewer drop-outs, and better results, when progress is measured (58 studies)

£4.70 / US$4

returned for every £1 / US$1 spent on mental-health support (Deloitte; WHO)

Year 5, our six countries (conservative)

~US$24m

of work time regained a year, from ~92,000 people in care

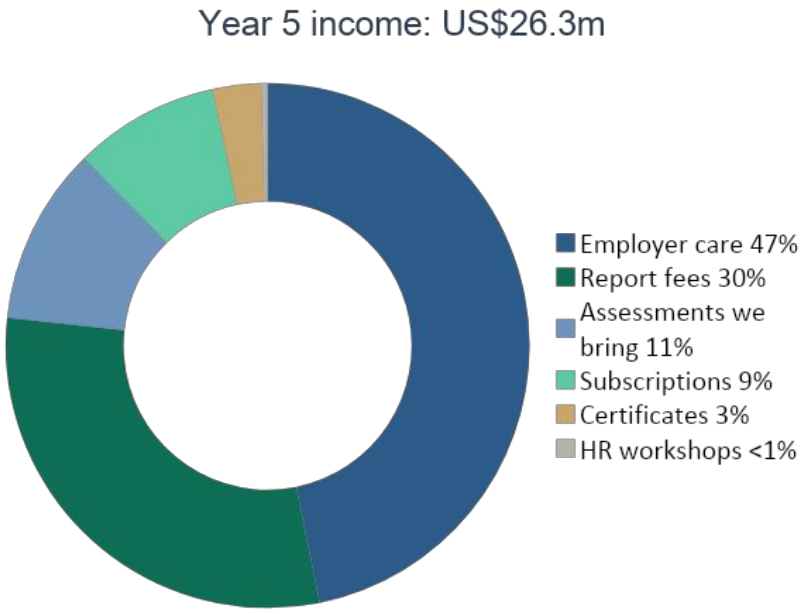
~1.9×

what employers pay us for care (US$12.4m)

Conservative: gains counted for only 3 months. If they last the year: ~US$95m (~7.7×). By country: Singapore 3.1×, Malaysia 2.2×, Philippines 2.1×, Thailand 1.2×, Indonesia 1.1× (lower wages).

Sources: Workplace Outcome Suite Annual Report 2020 (EAPA); de Jong et al. (2021); Deloitte (2024); WHO (2016). Our calculation with national wage data: Appendix A9.

Reason 2: employers start the growth; software drives profit



Income line	Who pays	Price	Share
Employer care	Employers	Monthly fee per employee	47%
Report fees	Psychologists	US$2 / 0.50 / 0.90 per report	30%
Assessments we bring	Employers, families, public	By type and country (we keep 30%)	11%
Subscriptions	Psychologists	US$30 a month	9%
Certificates	Employers or government	~US$300	3%
HR workshops	Employers	15% commission	<1%

Prices by country (US$)

	Care / employee / month	Signed assessment
Singapore	7.00	150
Malaysia, Thailand	2.00	60
Indonesia, Philippines, Vietnam	0.91	45

Employers pay less

11% less in Indonesia, 22% in Singapore and 35–52% elsewhere than if every case saw a psychologist

Psychologists paid in full

Full rate for care hours and 70% of each assessment fee; the 15% family discount comes from our share

Signed reports, not cheap tests

Psychologist-signed reports sell for US$30–120 in the market; low-cost online tests can be upgraded

Software: 57% of gross profit

Only 39% of income, but high margins: subscriptions 85%, report fees ~71% (assessments ~41%, care 32%)

Per-country margins and the Indonesian worked example: Appendix A1–A2. Published market prices: Appendix A3.

Reason 2: psychologists join because the workspace pays them back

Singapore — Year 1

220

subscribers in Year 5: 40% of ~550 psychologists (target 60%)

~50% of all reports on our platform (target ~70%)

Indonesia — Year 2

5,600

subscribers in Year 5: 40% of ~14,000 psychologists (target 60%)

~50% of all reports on our platform (target ~70%)

Thailand — Year 3

415

subscribers in Year 5: 40% of ~1,037 psychologists (target 60%)

~50% of all reports on our platform (target ~70%)

Philippines, Malaysia, Vietnam

224

subscribers: up to 10% of ~2,240 psychologists. Employer work comes first here.

How we win psychologists

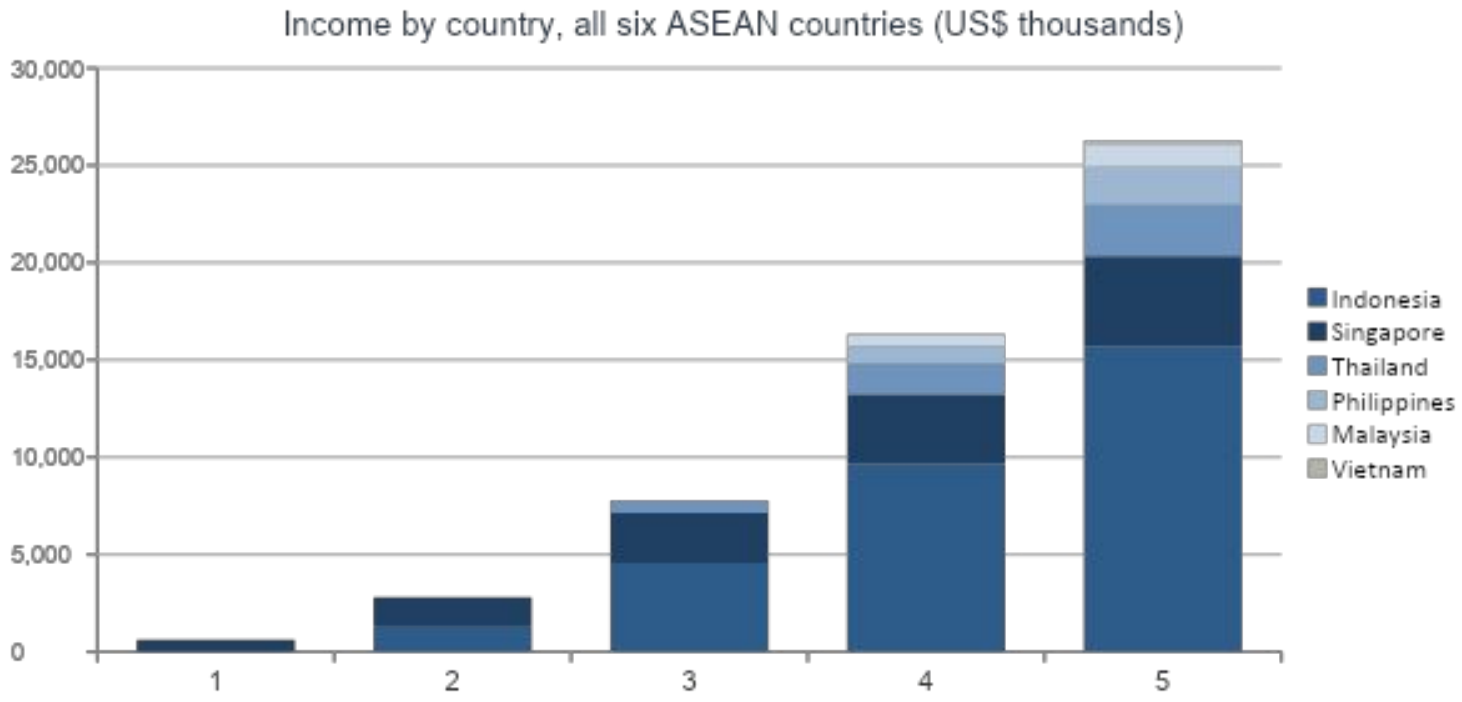
- **Paid work from employers** (the catalyst)
- Free trial; report fees from only **US$0.50**
- Partnership with **HIMPSI** and other associations, with continuing-education credits
- Psychology students trained on the platform bring it with them

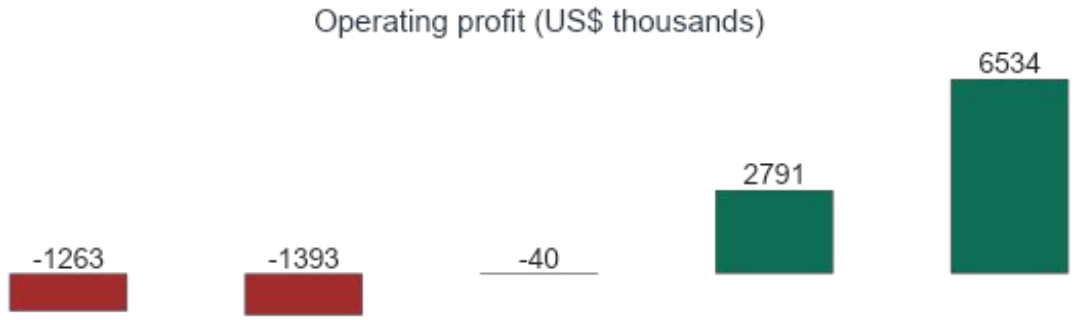
Why report share is higher than subscriber share

- The busiest psychologists join first
- A subscriber writes ~30 reports a week on the platform (half in their first year); others write ~20
- Target case: 60% of psychologists at ~35 reports a week gives ~70% of reports

Indonesia, Thailand and Singapore: more money for, and more attention to, mental health. We plan on the cautious base case. Assumptions: Appendix A5.

Reason 2: break-even in Year 3, profit from Year 4





Year 3

break-even (operating profit about zero); losses repaid in Year 4

US$6.5m

profit in Year 5 on US$26.3m income (25% margin)

US$2.7m

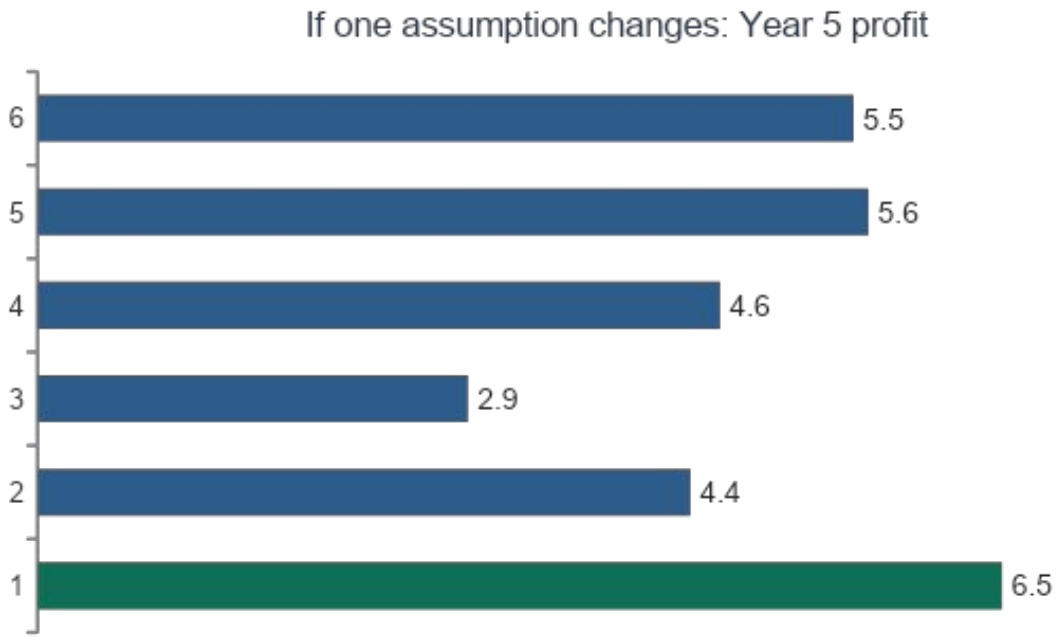
peak total losses (end of Year 3)

US$3.5m

to raise: US$1.5m now + US$2.0m only if the pilot meets its targets

Base case, all six ASEAN countries. Year 5 income: Indonesia 60%, Singapore 18%, Thailand 10%, Philippines 7%, Malaysia 5%, Vietnam 1%. Employees covered: 6k, 65k, 195k, 440k, 705k. Target case (60%): Year 5 profit US$11.8m (35%). Full plan: Appendix A4.

Reason 2: how sure are we? The plan survives the main risks



In every case the business makes a profit in Year 5.

The worst cases lose up to US$4.1m, up to US$0.6m more than the US$3.5m we raise. We cover the difference by slowing hiring until revenue arrives, or with a small bridge from the second-round investors.

What could go wrong, and our response

Everything one year late

Only add running costs when contracts and subscribers arrive: break-even in Year 4, about US$4.1m needed (US$7.1m if costs are not cut).

More employees use care than planned

The fee covers planned use; above that, a per-case fee at cost + 30% (about US$45 per mild case in Indonesia).

Singapore pilot misses its targets

The second US$2.0m is not invested; losses stop at about US$1.3–1.5m.

Reason 3: Singapore as the beachhead, ASEAN next, then the world

Rule for choosing markets: a big gap (too few psychologists) + employers who can pay + laws that allow supervised support roles.

1. BEACHHEAD • Year 1

Singapore

Strictest AI and data rules: pass here, pass anywhere. ~4,200 regional HQs: one contract reaches staff across ASEAN.

2. REGION • Years 2–5

ASEAN

80.4m people with a mental disorder; ~3 psychologists per 100,000; 5th-largest economy (US$3.8 trillion). ID → TH → PH, MY → VN.

3. WORLD • from Year 6

Same gap, same model

Other middle-income regions, starting with South Asia, through partners; high-income countries as software only.

How we become number one: we own five layers, and each one feeds the next

5 Standard	Local test norms: a score only means something against a local reference group, which takes years of research per country. Our test volume builds ASEAN norms fastest, with universities and associations.
4 Proof	Before-and-after results in work hours regained: the way employers measure what care returns
3 Network	More psychologists → more capacity for employers → more paid work → more psychologists
2 Intelligence	The checked knowledge library improves with every correction (~7.2m signed reports a year by Year 5)
1 Workflow	The psychologist's daily workspace: once a practice runs on it, it stays

A good kind of dominance

- Psychologists own their client data and can export it at any time
- Simple, fair fees agreed with associations: US$30 a month
- Open: other test publishers and clinics can plug in
- Power moves to small psychologists, not to a few AI companies that replace them

Sources: ASEANstats (2024); Bloomberg Intelligence via BritCham Singapore (2024); GBD 2021 ASEAN (2025). Safety rules stay the same everywhere; staff, language and price adapt (A6). Every Year 5 target needs at most 7% of a country's psychologist time (A7).

Risks and safeguards: AI is never the decision-maker

AI RISK

AI makes errors in scores or reports

→ A psychologist checks every score and signs every report; the assistant shows the source behind each statement; corrections improve the library.

AI RISK

Psychologists trust the AI too much

→ The assistant asks questions before drafting and marks statements needing a closer look; monthly sample checks of signed reports.

AI RISK

AI misses someone in danger

→ AI can only raise the level of care; a person reviews every warning the same day; emergencies go straight to crisis services.

CLINICAL RISK

Career tools used for clinical conclusions

→ DISC and RIASEC are for work and development only; mental-health conclusions use validated clinical questionnaires and a psychologist.

TRUST RISK

Employers misuse private information

→ Anonymous group results only (except recruitment tests with consent); written into every contract.

TRUST RISK

Psychologists or associations object to AI-drafted reports

→ The psychologist leads and signs every report; we agree ethical rules with HIMPSI and other associations and offer continuing-education credits.

TRUST RISK

Conflict of interest in our evidence

→ Pilot data comes from a company co-founded by a team member; we say so openly and the Singapore pilot tests it independently.

Recommendation: fund the Singapore pilot, then scale step by step

1. Invest **US$1.5m** in a one-year Singapore pilot with partner psychologists and regional employers.

2. Make the workspace every psychologist's daily tool: paid work as the catalyst, a free trial, report fees from **US$0.50**, the **70/30 split** and full pay for care hours.

3. Build the support workforce as a source of new jobs, including **workers who lost jobs to AI**, with government training schemes.

Targets before the next US$2.0m

Area	Target after 12 months
Report quality	≥90% of AI-drafted reports need only minor changes (blind review)
Diagnosis accuracy	Agreement ≥0.6 ('good') with an independent senior psychologist
Time saved	Psychologist time per full assessment at least halved
Psychologist income	Partner psychologists earn more per month than before
Safety	Every emergency warning reviewed on time; no serious harm caused by AI
Customers	≥3 employers renew; ≥1 extends to its Indonesian staff
Psychologist sign-ups	≥60 Singapore psychologists (~10%) pay for the workspace
Legal	Written legal opinion: support roles allowed in Indonesia

Too few psychologists is not only a training problem. It is a problem of how their time is used.

When AI does the scoring and drafting and a psychologist makes every decision, psychologists earn more, clients are understood correctly and get the right help, and employees become more productive. Used this way, AI creates jobs instead of removing them.

Augment, don’t replace.

Group 9

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Declaration on the use of AI

Done by our team

- Chose the topic and the business idea
- Built the main argument: assessment is the slowest and worst-paid work, so problems are misunderstood
- Made every key decision: business model, prices, 70/30 split, market order, family policy
- Prepared the first draft and first calculations, and supplied the pilot data
- Reviewed, debated and accepted or rejected every AI suggestion

AI assistance (about 20%, team estimate)

- Tool: Claude (Anthropic), used as a reviewer and editor
- Checked the consistency of our argument across sections and challenged weak points in discussion
- Checked and re-ran our financial calculations from our assumptions
- Found and checked sources, and published market prices
- Improved the English, the structure and the diagrams

The team is responsible for all content, numbers and conclusions in this proposal.

Appendix

A1 Care economics by country • A2 Indonesia worked example • A3 Published assessment prices • A4 Five-year plan • A5 Calculations and assumptions • A6 Country rules • A7 Psychologist supply check • A8 ASEAN at a glance • A9 Productivity regained • A10 Why big AI companies cannot easily enter • A11 Market size: B2B first, B2C later

A1. Care economics by country (per 1,000 employees per year, US$)

Country	Price level	Our income	Direct cost	Gross margin	Psychologist-only cost	Employer saves
Singapore	High (US$7.00)	84,000	50,900	39%	108,000	22%
Malaysia	Middle (US$2.00)	24,000	16,600	31%	37,100	35%
Thailand	Middle (US$2.00)	24,000	19,500	19%	49,600	52%
Indonesia	Base (US$0.91)	10,900	6,800	37%	12,200	11%
Philippines	Base (US$0.91)	10,900	9,300	14%	20,500	47%
Vietnam	Base (US$0.91)	10,900	9,600	12%	21,000	48%

- Assumes 10% of employees need light support (5 sessions) and 3% need psychologist care (6 sessions); 1 hour of supervision per 10 light-support sessions.
- Psychologists paid their normal hourly rate in full: S$180, RM200, THB2,100, Rp259,000, PHP1,500, VND700,000. Support workers: S$4,700, RM4,200, THB28,000, Rp9m, PHP32,000, VND15m per month (100 sessions).
- “Psychologist-only cost”: all 13% see a psychologist for six sessions at the lowest local private fee. Gross margin = (income − direct cost) ÷ income.

A2. Worked example: care for 1,000 employees in Indonesia for one year

Item	How we calculate it	Rp million
Income	1,000 employees × Rp15,000 × 12 months	180.0
Light support	500 sessions × Rp90,000 (support worker Rp9m a month, 100 sessions)	45.0
Supervision	50 hours × Rp259,000 (psychologist's normal hourly rate)	13.0
Psychologist care	180 sessions × Rp259,000	46.6
AI tools	Rp8,000 per employee per year	8.0
Gross profit	**180 − 112.6 total direct cost**	**67.4 (37%)**

- If the same 13% all saw a psychologist: 780 sessions × Rp259,000 = Rp202 million. We charge Rp180 million (11% less), including check-ins for everyone else.
- A mild case costs us Rp525,000–580,000 vs Rp1.55 million for six private sessions. Above planned use, employers pay per case at cost + 30% (about Rp750,000).
- US company helplines cost about US$12–40 per employee a year, but only about 5% of staff use them; our base price is US$10.91 a year.

APPENDIX

A3. Published assessment prices (examples)

Market	Service	Published price	Source
Indonesia	Online test, automatic report	From Rp60,000 (promotions from Rp25,000)	Satu Persen (2026)
Indonesia	Online staff recruitment tests	Rp50,000–150,000 per candidate	KantorKu (2026)
Indonesia	IQ / interests and aptitude test	Rp100–400k / Rp250–500k	Solutiva (2026)
Indonesia	Job selection test with consultation	Rp300,000–400,000	UKSW
Indonesia	Manager and director selection	Rp900,000–2 million	Astagina Praptama (2022)
Indonesia	School readiness with psychologist	Rp800,000–1.5 million	Bank Jago
Indonesia	Assessment centre	From Rp1.5 million	Solutiva (2026)
Global	CliftonStrengths 34 (no psychologist)	US$59.99	Gallup
Singapore	Career / clinical personality / IQ test	S$200 / S$500 / S$800	Rogerian Psychology
Singapore	Full educational / ADHD assessment	S$2,800–4,000 / from S$2,673	Rogerian; Dyslexia Assoc. of Singapore
Malaysia	ADHD assessment (private)	RM1,300–2,600	Lucent Wellbeing

Prices published by providers, not an independent market survey. Our focus: psychologist-interpreted reports (Rp500,000–2 million band).

A4. Five-year plan (US$ thousands)

Year	Countries	Employees covered	Psychologists subscribed	Income	Gross profit	Running costs	Operating profit	Margin	Cumulative
1	Singapore	6,000	28	605	242	1,505	−1,263	−209%	−1,263
2	+ Indonesia	65,000	502	2,792	1,298	2,692	−1,393	−50%	−2,657
3	+ Thailand	195,000	1,869	7,781	3,847	3,887	−40	−1%	−2,697
4	+ Philippines, Malaysia	440,000	3,960	16,307	8,091	5,300	2,791	17%	+94
5	+ Vietnam (small)	705,000	6,459	26,269	13,227	6,693	6,534	25%	+6,629

- Year 5 scale: 6,459 psychologists subscribed (Indonesia 5,600; Thailand 415; Singapore 220; Philippines 170; Malaysia 40; Vietnam 14), ~7.2 million reports a year on the platform (about half of all reports in the 3 core markets), 290 Wellbeing Practitioners, ~140 assistants, ~373,000 assessments we bring.
- Year 5 income: care 47%, report fees 30%, assessments 11%, subscriptions 9%, certificates 3%, HR <1%. US$12.4m paid to psychologists for work we bring. Target case (60% subscribed, 35 reports a week): income US$33.8m, profit US$11.8m (35%).
- Running costs include Singapore staff, clinical leadership, the AI and knowledge library, recruiting psychologists, data protection, sales and insurance.

A5. Calculations and main assumptions

- **Care per 1,000 employees:** 10% × 5 = 500 support sessions; supervision 500 ÷ 10 = 50 h; psychologist care 3% × 6 = 180 h; total 230 psychologist hours vs 780 if all 13% saw a psychologist.
- **Psychologist income (Indonesia):** today 6 × 4.33 × Rp700k = Rp18.2m/month for ~63 h/week. Platform: 15 × 4.33 × Rp490k = Rp31.8m − US$30 = Rp31.3m for ~33 h/week (2.2 h each = 10 min score check + 1 h review and sign + 1 h feedback).
- **Assessments:** 15% of covered employees get a signed assessment each year; recruitment tests equal to 15% of covered employees; family assessments 1% (15% off, paid from our share); 10% of public online-test buyers upgrade.
- **Direct cost per assessment:** assistant 1.5 h (full) or 15 min (standard test) at local pay, plus AI and test licences US$2.50 (full) or US$0.70 (standard).
- **Software (base):** share subscribed, Years 1–5: Singapore 5/15/25/33/40%; Indonesia 0/3/12/25/40%; Thailand 0/0/5/20/40%; others up to 10%. 30 reports a week × 46 weeks (half in year one). Mix: 40% with our tests (US$2, cost US$0.60), 45% report-only ≤3 tests (US$0.50, cost US$0.15), 15% report-only >3 tests (US$0.90, cost US$0.20). US$150 to win a subscriber, US$40 support a year, 10% replaced a year.
- **Other:** subscription US$30/month (15% direct cost); no report fee on work we bring; certificates US$300 (45% direct cost); HR bookings US$1 per covered employee per year at 15% commission.
- **Exchange rates per US$:** S$1.30, RM4.20, THB33, Rp16,500, PHP57, VND26,000.
- **Evidence status:** time savings and quality ratings are internal pilot data (Coreitera); the Singapore pilot will test them independently.

A6. Country rules and data laws

Country (year)	Price level	Who can be a support worker	Data law	Why this order
Singapore (1)	High	Counselling not licensed by law; psychologist titles to be protected, so we avoid them	PDPA 2012	Proves safety; regional buyers; high prices; core software market
Indonesia (2)	Base	Law 23/2022 lets psychology graduates assist psychologists	PDP Law 2022	Largest shortage; ~14,000 psychologists; core software market
Thailand (3)	Middle	Legal opinion first	PDPA 2019	Core software market; free public care, so we offer speed, privacy and assessments
Philippines (4)	Base	“Guidance counselor” and “psychologist” protected; legal opinion first	Data Privacy Act 2012	Call-centre workers can retrain; English
Malaysia (4)	Middle	Counselling licensed; we hire registered counsellors	PDPA 2010 (updated 2024)	Regional buyers
Vietnam (5)	Base	Health licensing tightened in 2024; to be checked	PDP Law (2026)	Only ~143 clinical psychologists: small partner-led entry

Sources: MOH Singapore (2026c); UU 23/2022; UU 27/2022; RA 9258; Counsellors Act 1998; Tilleke & Gibbins (2025). Culture: privacy wall, check-ins for everyone, locally validated tests, faith and family options.

A7. Growth is limited by psychologists, not employers: supply check

Country	Psychologists today	Per 100k	Subscribed, Y5 (base)	Employees covered, Y5	Full-time psychologists needed for work we bring
Indonesia	~14,000 (3,059 clinical); 300,000+ psychology graduates	5.0	5,600 (40%)	420,000	283 (2% of national)
Thailand	~1,037	1.6	415 (40%)	70,000	47 (5%)
Singapore	~550 registered	9.7	220 (40%)	45,000	30 (6%)
Philippines	~1,700 licensed (~800 clinical); thousands of licensed psychometricians	1.5	170 (10%)	120,000	81 (5%)
Malaysia	~400 clinical psychologists; 9,562 registered counsellors	1.1	40 (10%)	40,000	27 (7%)
Vietnam	~143 clinical psychologists and psychotherapists	0.1	14 (10%)	10,000	7 (5%)

- **Every target passes:** the work we bring needs at most 7% of any country's psychologist time; subscribers use the workspace mostly for work they already do.
- **Indonesia is the core market** (60% of covered employees and 87% of subscribers in Year 5). **Vietnam stays small:** the earlier target of 40,000 employees would have needed about a fifth of all its clinical psychologists.
- Philippine psychometricians may legally give and score tests under a psychologist's supervision: a ready assessment-assistant workforce.

Sources: HIMPSI (2026, 2021); IPK; Inquirer (2024); MOH Singapore (2021); The Star (2025); CodeBlue (2023); New Mandala (2023); Vietcetera (2024).

A8. ASEAN at a glance: the burden behind burnout

Country	People with a mental disorder (2021)	Share of population (age-standardised)	Employees with high burnout	Psychologists	People with a disorder per psychologist
Indonesia	32.9 million	11.8%	Lower than Malaysia	~14,000	~2,350
Philippines	14.0 million	12.9%	71%	~1,700	~8,200
Vietnam	10.1 million	10.1%	Not measured	~143 (clinical)	~70,600
Thailand	8.9 million	12.8%	Not measured	~1,037	~8,600
Malaysia	4.1 million	13.2%	58%	~400 (clinical)	~10,300
Singapore	0.65 million	12.3%	67%	~550	~1,200
Our 6 countries	70.7 million	~12%	63% (4 countries)	~17,800	~4,000
All ASEAN (10)	80.4 million	11.9%			

- **Assessment capacity (our estimate):** ~17,800 psychologists × ~5 hours a week on assessment ÷ 10.5 hours per full assessment × 46 weeks **≈ 0.4 million full assessments a year**, about **0.6%** of the 70.7 million. Even with every psychologist doing only assessments, it would be about 7%.
- **Care reached:** at most 1 in 3 people with depression receive formal care worldwide (WHO); in Indonesia, 10.4% of young people with depression received treatment (SKI 2023).
- These counts cover mental disorders only. Burnout and other work and life problems add more demand on top.

Sources: GBD 2021 ASEAN Mental Disorders Collaborators (2025), Lancet Public Health; Aziz & Ong (2024); WHO (2025) Mental disorders fact sheet; Kemenkes (2024) SKI 2023; supply data from A7.

A9. Productivity regained: calculation by country (Year 5)

Country	Average monthly wage	US$ per hour	People in care (13%)	Regained, 3 months (US$m)	Employer care fee (US$m)	Return per US$1	If gains last 12 months
Singapore	S$5,500	24.41	5,850	11.6	3.8	3.1	12.3
Malaysia	RM3,652	5.02	5,200	2.1	1.0	2.2	8.9
Philippines	PHP21,544	2.18	15,600	2.8	1.3	2.1	8.5
Vietnam	VND8.2m	1.82	1,300	0.2	0.1	1.8	7.1
Thailand	THB15,500	2.71	9,100	2.0	1.7	1.2	4.8
Indonesia	Rp3.27m	1.14	54,600	5.1	4.6	1.1	4.4
Total			**91,650**	**23.8**	**12.4**	**1.9**	**7.7**

- **Formula:** people in care × 27.1 hours regained a month × months counted × hourly wage. 27.1 hours = lost productive time falling from 63.2 to 36.1 hours a month in the Workplace Outcome Suite data (35,693 cases).
- **People in care:** 10% light support + 3% psychologist care of 705,000 covered employees. Hourly wage = monthly wage ÷ 173 hours; exchange rates as in A5.
- **Why conservative:** gains counted for 3 months only; employer benefits from lower staff turnover and correct diagnosis are not counted. Self-reported hours can overstate gains, so the pilot compares teams that start earlier with teams that start later.

Wages: MOM Singapore (2024 median incl. employer CPF); DOSM (2024 mean); PSA (2024); GSO Vietnam (Q4 2024); NSO Thailand labour force survey (approx.); BPS (Aug 2024).

A10. Why ChatGPT, Claude or Grok cannot easily take this market

1	**Licence and liability**	Only a licensed psychologist can sign a report, and the law wants a person responsible (Singapore MOH; Illinois 2025). A general AI company would carry clinical liability in every country.
2	**Local norms and tests**	Tests belong to publishers, and scores need local norms built by years of research. A general model has neither. Our data: 4.2/6 without the checked library, above 5.5/6 with it.
3	**Data they cannot reach**	Clinical data is protected by data-protection laws and lives inside the psychologist's workflow. It cannot be scraped from the web; we hold it with consent.
4	**Distribution and trust**	Employers buy through HR contracts, local psychologists, associations and support staff on the ground. A chat app has none of these.
5	**Focus and brand risk**	For them, ASEAN assessment is a small, high-risk niche. One harmful chatbot answer damages a global brand (a US helpline chatbot was shut down in 2023).

More likely our suppliers than our rivals

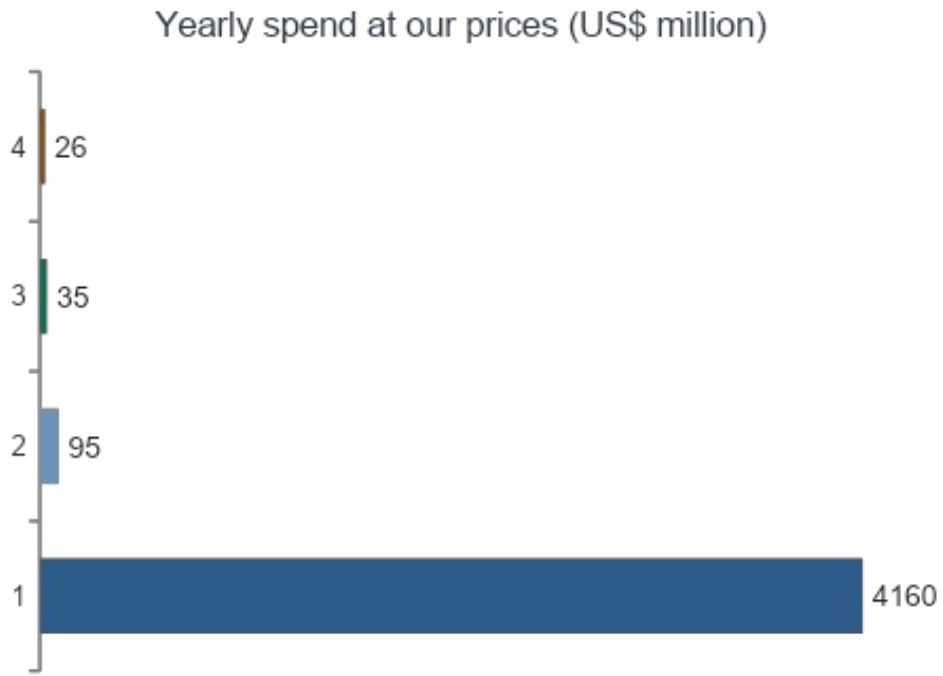
We use their models underneath and can switch between them. Our value is not the model: it is the five layers on top (workflow, checked library, network, proof, local norms).

What we watch

A big HR or health-record platform teaming up with an AI company. Our answer: open connections for partners, and norms built together with associations.

Sources: MOH Singapore (2026a); IDFPR (2025); CNN (2023); our pilot data (205 reports rated over 6 months).

A11. Market size: far larger than our plan; B2B first, B2C after Year 3



- **B2B:** care (US$2.5bn) and assessments (US$1.7bn) at our prices for every employee in our six countries ≈ US$4.2bn a year. In Year 5 we cover 705,000 employees: about 0.4% of them.
- **B2C (illustrative, not in our plan):** if 1 in 10 of the 70.7m people with a mental disorder took one US$9 online test a year, and 10% upgraded to a US$45 full assessment ≈ US$95m.
- **Beyond ASEAN** (from Year 6) is not counted at all.

Why B2B first (Years 1–3)

	B2B (employers)	B2C (individuals)
Winning clients	One contract covers thousands of staff	One person at a time
Managing problems	A known group, an HR contact and a clear escalation path	Anonymous users; harder to reach in a crisis
Data quality	Consented, repeated check-ins: good for norms and proof	Patchy, one-off data
Blame and damage control	Shared with the employer under a contract	Public and fast; one mistake spreads widely

When we open B2C: after Year 3, and only when the safety, accuracy and before-and-after targets are proven at scale. Until then, B2C is limited to low-cost online tests that lead to a psychologist (about 2% of Year 5 income).

Employees = labour force × share of wage and salaried workers (World Bank / ILO modelled estimates, 2025). Prices as in A1 and A5. B2C line is our illustration only.

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